

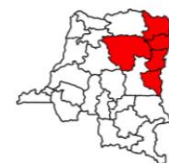


Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°089/MVEBDB/11/08/2026

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**HIGHLIGHTS** (last 24 hours)

In the last 24 hours, 118 new confirmed cases and 67 deaths (50 community deaths and 17 intra-Ebola Virus Disease (EVD) cases) have been reported in the provinces of Ituri (98 cases) and North Kivu (18 cases) and Haut Uélé (2).

ACCUMULATIVE CASE 4,566	ACCUMULATIVE CONFIRMED DEATHS 2,128	LETHALITY 46.6%	PATIENTS IN ISOLATION / CTE 570	ACCUMULATIVE HEALED 918	TRACKING RATES CONTACTS (Of the Day) 82.4%
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Affected Provinces and Affected Health Zones

**5 Provinces Affected**

Haut-Uélé, Ituri, North Kivu,
South Kivu & Tshopo

**53 Health Zones**

Affected Out of
140 (37.9%)

**246 Health areas affected**

Out of 3104 (7.9%)

Summary of key points from the last 24 hours

In the last 24 hours, 118 new confirmed cases and 67 deaths (50 Community deaths and 17 Ebola Virus Disease (EVD) deaths within Ebola Treatment Centers (ETCs) were reported in the provinces of Ituri (98 cases), North Kivu (18 cases), and Haut-Uélé (2 cases). This is the highest daily number of confirmed cases since the start of the outbreak, with Ituri doubling its incidence in 24 hours.

To date, the Democratic Republic of Congo has recorded 4,566 confirmed cases, including 2,128 deaths, representing a case fatality rate of 46.6%. Ituri remains the epicenter of the epidemic, accounting for 85.7% of cumulative confirmed cases and 83.1% of new cases reported in the last 24 hours.

Thirty (32) patients were declared cured after obtaining two negative results for Ebola Virus Disease (EVD) (29 in Ituri and 2 each in North Kivu and Tshopo). Conversely, 17 deaths within Ebola Treatment Centers (ETCs) were recorded in Ituri (10), North Kivu (6) and Haut-Uélé (1).

The epidemic affects 53 of the 140 health zones in the five affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23) and South Kivu (1/34).

Distribution of confirmed cases and deaths by affected province

(As of August 11, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Health Zones touched	New confirmed cases (24h)
Ituri	3,912	1702	43.5%	28/36 (77.8%)	98
North Kivu	528	369	69.9%	12/34 (35.3%)	18
Haut-Uélé	115	51	44.3%	6/13 (46.2%)	2
Tshopo	9	5	55.6%	6/23 (26.1%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	4,566	2,128	46.6%	53/140 (37.9%)	118

Confirmed cases and deaths by province and health zone (situation as of August 11, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3912	1702	43.5%	98	39	10	49
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	7	2	28.6%				0
Aungba	10	5	50.0%				0
Bamboo	93	22	23.7%				0
Boga	1	1	100.0%				0
Bunia	1072	329	30.7%	29	12		12
Damascus	20	9	45.0%	2	1		1
Drodro	12	6	50.0%				0
Fataki	55	29	52.7%				0
Gethy	2	1	50.0%				0
Kambala	2	0	0.0%				0
Kilo	30	12	40.0%	1	1		1
Komanda	42	29	69.0%				0
Lita	169	97	57.4%	6	1		1
Logo	11	5	45.5%				0
Lolwa	11	4	36.4%				0
Mahagi	4	1	25.0%				0
Mambasa	13	6	46.2%				0
Mandima	19	13	68.4%				0
Mangala	120	76	63.3%	4	2		2
Mongbwalu	579	281	48.5%	3	1		1
Nia-Nia	146	87	59.6%	4	3		3
Nizi	502	231	46.0%	13	4		4
Nyankunde	117	35	29.9%	1	0		0
Rimba	9	4	44.4%				0
Rwampara	800	316	39.5%	35	14		14
Tchomia	48	24	50.0%				0
To be ventilated	N / A	69	N / A			10	10
North Kivu	528	369	69.9%	18	9	6	15
Blessed	80	62	77.5%	2	1	1	2
Butembo	105	86	81.9%	2	1		1
Goma	1	0	0.0%				0
Kalunguta	10	4	40.0%				0
Katwa	249	166	66.7%	10	6	4	10
Kyondo	13	10	76.9%				0

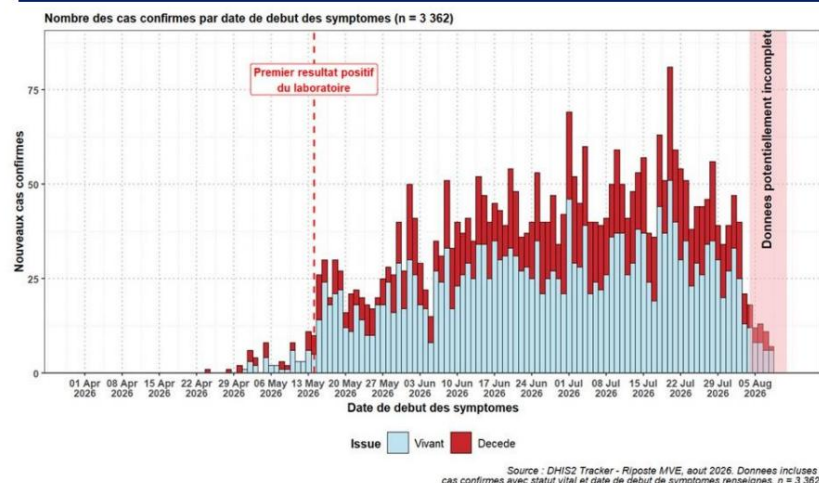
Lubero	1	1	100.0%				0
Mabalako	1	0	0.0%				0
Masereka	7	4	57.1%				0
Musienene	53	31	58.5%	4	1	1	2
Oicha	5	4	80.0%				0
Vuhovi	3	1	33.3%				0
Haut-Uélé	115	51	44.3%	2	2	1	3
Boma Mangbetu	20	5	25.0%				0
Gombari	1	1	100.0%				0
Isiro	33	18	54.5%	2	2		2
Pawa	17	11	64.7%				0
Rungu	1	0	0.0%				0
Wamba	43	16	37.2%		1S	1	2
Tshopo	9	5	55.6%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	2	1	50.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	3	3	100.0%				0
Mangobo	1	1	100.0%				0
Wanie-Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Total	4566	2128	46.6%	118	50	17	67

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

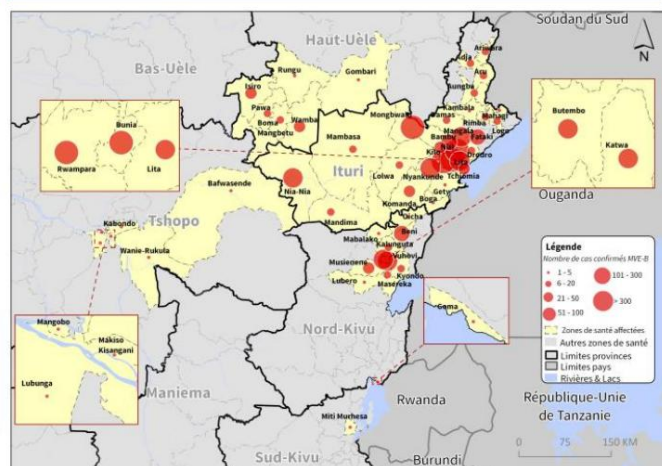
Status of alerts notified by province, as of August 11, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	7	3	10	7	3	0	0	10	6
Ituri	516	52	568	109	51	288	1	160	95
North Kivu	885	49	934	100	49	785	0	148	89
South Kivu	0	0	0	0	0	0	0	0	0
Tshopo	29	1	30	5	1	24	0	6	5
Total General	1437	105	1,542	221	104	1097	1	324	195

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Support for the Beni zonal coordination and continuation of the mission support for the Butembo health zone.

1.2. Epidemiological surveillance

1.2.1. Main Actions

- In total, 1,542 alerts were recorded, of which 1,423 (92.2%) were verified within 5 provinces affected for the day of August 11, 2026. However, 324 (99.7%) were validated as suspected cases and all were investigated, of which 195 suspects were transferred to CT/CTE.
- The proportion of contacts followed up in the last 24 hours is at 82.4% (17,085 seen out of 20,728 to be followed up), close to the threshold of 85%, with lower performance in North Kivu (60.7%).

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Six (6) alerts were detected at the various PoE/PoC (3 in Ituri and 3 in North Kivu): the three live alerts in Ituri were notified and validated at the PoCs of Basanjasili, Banguba, and Mudzibala, with two of them being referred to the CTE (66.6%) and one refusing transfer; in North Kivu, one live alert and two deceased individuals were intercepted and 100% managed. No alerts were notified in Haut-Uélé or Tshopo.
- In total, 209,068 travelers were screened (113,406 in Ituri, 72,963 in North Kivu, 18,531 in Haut-Uélé, and 4,168 in Tshopo), with a screening rate of 97%. In Ituri, only one of the 11 strategic PoE/PoC sites (9.1%) operated without interruption, and the completeness rate reached 55/60 (91.7%); in North Kivu, all 18 sites reported (100%) with 2,100 screening refusals (2.9%); in Tshopo, 6 of the 19 PoC sites were activated (32%), with a completeness rate of 57%.
- In Haut-Uélé, 10 PoE/PoC are activated out of the 38 priority sites (26.3%), 7 out of 10 sites have transmitted their data (70%) and 18,478 out of 18,531 travelers have been screened (99.7%);

The BOH PoC alone accounts for 4,171 travelers (23%). The pillar team conducted a field visit to the Wamba health zone to activate the Salisa PoC.

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: functionality remains low in Tshopo, with only 6 out of 19 (32%) activated, and reporting completeness is limited to 57%. • Constraints affecting PoE/PoC functionality: performance continues to be affected by refusals to screen (2,100 people, or 2.9%, in North Kivu), attacks on service providers, and non-compliance with health protocols by some authorities at the Mukulia PoC (North Kivu), as well as by a lack of transportation for supervision in Tshopo; 48 refusals to screen (1.2%) were recorded, motivated by denial of the disease and fear of being hospitalized at the Ebola Treatment Center (ETC).

1.4. Laboratory

1.4.1. Main actions

- Ituri recorded **119** positive results, including **98 new cases (59 live and 39 deaths)** and **21** repeat tests, while **North Kivu confirmed 18 cases, including 9 deaths, and Haut-Uélé, 2 confirmed cases out of 3 samples analyzed.** In total, **516 samples** were analyzed, confirming **118 new cases** of Ebola virus disease (positivity rate: **22.9 %**); Tshopo has not
No positive results were recorded.

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1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 19/73 rings were opened; 80/83 sites identified and decontaminated (96.3%), including 66 households and 11 health establishments and 3 churches; 59 EDS carried out out of 64 death alerts (92.1%) and in care establishments, and 15 EDS out of 15 in CTE, for 42 bodies collected post-mortem (72%).
- **In North Kivu**, 33 bodies were collected and secured and 31 EDS carried out, with 2 failures linked to community resistance in Kitovo and Butuhe; 34 death alerts were recorded, including 22 in healthcare facilities (64.7%), 7 in the community (20.6%) and 5 in CTEs (14.7%).
- **In Tshopo**, IPC activities were carried out in 4 health zones (4 rings opened, 4 households and 2 establishments decontaminated) and a lifeless body was prepared for EDS at the HC of Kisangani.
- **In Haut-Uélé**, three death alerts were recorded (two in Isiro and one in Wamba) for two death assessments (66.7%) and three bodies collected by swab; decontamination and death assessment activities are paralyzed in the Wamba health zone following the zonal suspension of the Red Cross. Triage compliance was verified at the General Referral Hospital and the University Clinic in Isiro.

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 31 out of 33 ambulances allocated to the response were available, with 30 successful trips completed (100%). Ebola treatment centers (ETCs) recorded 52 admissions and 29 recoveries in the last 24 hours; occupancy reached 375 patients out of 845 beds (44.4%) specifically in Ituri, with the Ebola treatment centers in Rwampara, Bambu, and Nizi at capacity for confirmed cases.
- **In North Kivu**, 219 cumulative admissions in CTE/CT, for an intra-CTE mortality of 42%; 52 admissions of the day (4 confirmed and 48 suspected), 2 recovered and 6 intra-CTE deaths, for 176 patients in hospital (25 confirmed and 151 suspected).
- 19 patients are hospitalized there, 13 in South Kivu and 9 in Tshopo.

1.6.2. Challenges

Insufficient availability of referral resources (2 ambulances out of service in Ituri, no ambulances in 7 health zones in North Kivu, and no TCCs/ETCs in 4 health zones) and saturation of several TCCs/ETCs limit rapid access to care and isolation of suspected cases. **In Haut-Uélé** : the lack of standardized TCCs persists in the 6 affected health zones; insufficient mobility for community health workers in the Wamba, Pawa, and Boma Mangbetu health zones, the shortage of personal protective equipment in Pawa, and the unavailability of hygiene kits in all affected areas hinder the implementation of infection prevention and control measures.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 55 contacts were traced around 6 cases in six health areas of the Rwampara Health Zone; 1,797 people were reached during home visits, 121 during educational talks, and 5,486 during mass awareness campaigns; 22 radio programs were produced, and a batch of awareness materials was received from FHI360 for the Nia-Nia Health Zone.
- **In North Kivu**, formative supervision of the response was conducted in the ZS of Katwa, with support from the WHO, and 48 PoC providers from the Beni and Butembo pools were briefed on surveillance, IPC and CREC
- **In Tshopo**, 889 people were reached during home visits in the Makiso-Kisangani and Tshopo health zones, and 85 members of the management teams in 14 health zones were briefed on Ebola Virus Disease (EVD). In South Kivu, 50 people deprived of their liberty were trained in infection prevention and control (IPC) in the Nyangezi and Kadutu health zones, and 61 contacts were released from cantonment sites.

1.7.2. Challenges

Persistence of community challenges: announced strike of the psychosocial pillar and demotivation linked to non-payment in North Kivu, removal of a positive body by the community at the family cemetery of AS Misebere (ZS Katwa), refusal of dignified and secure burials (10 failures in Mangala and 4 in Komanda in Ituri) and refusal of collection in Tshopo, likely to affect the communities' adherence to the response.

1.8. Logistics

1.8.1. Main actions

In Ituri, the installed capacity reaches 845 beds (417 for confirmed cases, 428 for suspected cases); 189 cartons of F95 masks were received and 500 liters of fuel were distributed; 49 vehicles including ambulances are allocated to the response; the expansion of the CTEs in Bambu, Nizi and ISTM is requested in the face of saturation.

In North Kivu : Continued distribution of 1,200 liters of diesel and delivery of PCI inputs; continued construction of tank bases at the PoCs of Karuruma and Komba and of the triage unit of the Vuhovi General Referral Hospital (ULB Cooperation); packaging of nutritional inputs for seven health zones).

1.8.2. Challenges

Persistence of logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation.

1.9. Security 1.9.1.

Main actions

- In North Kivu, eight public order maintenance and restoration activities were carried out, including accompanying two burials of confirmed cases, securing the transfer of bodies to the morgues of Ngilika and Katwa and supporting swab operations.

1.9.2. Challenges

- Persistent insecurity, incidents involving local authorities and community resistance during dignified and secure burials, limiting access for response teams: physical assaults on service providers and law enforcement at the PoC in Mukulia, insecurity and broken suits blocking 10 EDS in Mangala.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the various communities of the affected provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES

Ebola transmission remains active, with 118 new cases and 67 deaths, including 50 fatalities community and 17 intra-CTE recorded in the last 24 hours, bringing the national total to 4,566 confirmed cases and 2,128 deaths.

Ituri remains the epicenter of the epidemic, concentrating 85.7% of cumulative confirmed cases and 83.1% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).

Surveillance has shown good performance over the past 24 hours, with 99.7% of suspected cases investigated, a follow-up rate of 82.4% close to the threshold of 85.0%.

Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. Kombe NGWAMA John

Editorial Team

Professor Dr. Mulangu Katulumba Félix-Médard

Dr. Katusi Biselenge Jules

Mayele Afua

With technical support from the World Health Organization (WHO), Africa CDC.

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd